HISTORICAL NOTE

JOHN CONOLLY AND THE INTRODUCTION OF NONRESTRAINT IN PSYCHIATRIC TREATMENT

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Just as the name of Phillipe Pinel is associated with the introduction of the humane treatment of institutionalized psychotics in France, so is the name of John Conolly remembered in English psychiatry as the man who introduced nonrestraint in England. John Conolly was born in Lincolnshire in 1794. While still in his teens he paid a visit to a mental hospital in Glasgow and was so impressed with what he saw there that he never afterwards ceased to take the deepest interest in abnormal mental and emotional behavior.

When he was eighteen Conolly became an officer in a militia regiment, in which capacity he served for several years. He married the daughter of Sir John Collins in 1816 and went to live in France. Very shortly thereafter he decided that medicine was to be his life's work and in 1817 became a student at Edinburgh University. After receiving his M. D. degree he began practice at Chichester where he remained a very short time. In 1823 he removed to Stratford-on-Avon, where he engaged in literary activity in addition to his medical practice. He wrote many of the articles as well as took part in editing the "Cyclopedia of Practical Medicine" as well as the *British and Foreign Medical Review.* At Stratford he became alderman and later mayor, established a public dispensary, and in addition became an enthusiastic student of Shakespeare. He later took an active part as chairman of the committee formed for the purpose of preserving Shakespeare's tomb.

In 1827 Dr. Conolly was appointed professor of the practice of medicine in London University, which appointment he held only for 4 years. Finding life as a London physician not quite to his tastes, in 1831 he decided to leave London for a country practice, and he settled in Warwick.

During all this time Conolly was intensely interested in mental diseases. He had studied psychiatry both abroad and at home, and had been for 5 years (while residing in Stratford) inspecting physician to the Lunatic Houses for the County of Warwick, an office that he resumed when he settled in Warwick. He had unsuccessfully proposed to the council of the University that he should give his pupils clinical instruction in psychiatry in one of the mental hospitals in London. "Thus," says Sir James Clark in his biography of Conolly, "clinical instruction in mental diseases was thrown back for thirty years in this country."

In 1830 Conolly published *An Inquiry Concerning the Indications of Insanity, with Suggestions for the Better Protection and Care of the Insane.* In this book he discussed the clinical manifestations of the various psychoses and indicated humane methods of care and treatment. He described the deplorable conditions found in mental hospitals at that time and the woefully inadequate methods of treatment. He proposed that the State assume the responsibility of treating mental diseases. He was among the first to state that mental illness was increasing because of the increased pressures and tempo of modern life.

His book was not received with too great enthusiasm. Unfortunately at that time the average physician and even those who specialized in the treatment of mental ailments knew very little about diseases of the mind, and in addition they cared very little. So Conolly was left to his country practice at Warwick, interrupted by one year's residence at Birmingham, till 1839, when he was appointed resident physician to the Middlesex County Asylum at Hanwell, at that time the largest in England. He had taken the opportunity of visiting the Lincoln Asylum and thoroughly surveyed conditions there. He was now convinced that mechanical restraint was not only unnecessary, but injurious. He succeeded in convincing others that his system was the right one. Hanwell had the reputation of being one of the best managed asylums in England, many patients being occupied in agricultural and other useful pur-

suits. Yet the year after Sir William Ellis's resignation, when Conolly took office, "instruments of mechanical restraint of one kind or another were so abundant in the wards as to amount, when collected together, to about six hundred, about half of them being handcuffs and leg-locks."

Conolly entered upon his duties June 1, 1839. The hospital then contained 800 patients, and he found 40 under continuous mechanical restraint. In his first report to the Quarter Sessions, he informed the Justices that since the 21st of September not one patient had been under restraint. "No form of strait waistcoat, no handcuffs, no leg-locks, nor any contrivance confining the trunk, or limbs, or any of the muscles, is now in use. The coercion chairs, about forty in number, have been altogether removed from the wards." In fact, they had been cut up to make a floor for the carpenter's shop.

This was not accomplished without overcoming quite a few obstacles. It took time to indoctrinate the ward physicians and attendants. In 10 years not one case was admitted to Hanwell in which mechanical restraint was deemed necessary, although many suicidal patients were among them. "The great and only real substitute for restraint is invariable kindness," said Dr. Conolly. "This feeling must animate every person employed in every duty to be performed."

At the end of 10 years the nonrestraint system that he had introduced was so firmly established that Dr. Conolly felt he could resign as resident physician. He became visiting physician. His interest in the patients under his care was thorough. He would visit the wards at all hours of the night. He treated his patients with the greatest consideration, always looking for something to commend in a patient, the hair better kept, the clothes more neatly worn, encouraging the patient at the progress made toward recovery.

In 1852, on his resignation as visiting physician, Conolly's connection with Hanwell practically ceased, and a portrait of him painted by Sir W. Gordon was presented to him at a public meeting by Lord Shaftesbury.

His resignation from Hanwell did not mean that Dr. Conolly would resort to a life of ease. In fact, his activities increased. He aided in the establishing of several hospitals for mental diseases and mental deficiency and lectured quite extensively on psychiatric topics to medical men. In addition he had a large psychiatric practice, being recognized as one of the leading psychiatrists in England.

In 1856 he wrote one of the classics in English psychiatric literature, *The Treatment of the Insane Without Mechanical Restraint.* This was widely read and proved a great influence in altering the treatment of the mentally ill along more humane lines. Also, for a number of years he was a very popular lecturer at the College of Physicians and at the Royal Institution, his courses in psychiatry being well attended by physicians who helped to spread his doctrines into almost all of the English mental hospitals. He was among the first psychiatrists to write on child psychiatry, and his papers on this subject attracted a great deal of favorable attention.

As mentioned above Dr. Conolly became interested in Shakespeare quite early in his career, and he was the first psychiatrist to write a psychiatric evaluation of Hamlet. His paper was called *A Study of Hamlet,* in which he sought to prove that Hamlet was not feigning insanity, but was actually suffering from a definite psychosis. As to Hamlet's treatment of Ophelia in Act II, Scene I, and more especially in the scene where Hamlet and Laertes met over her grave, he says: "The picture of madness here is too minutely true, its lights and shades are too close to nature to have been painted as a mere illustration of feigning, and of feigning without intelligible purpose."

In his late sixties Conolly was subjected to attacks of neuralgia and chronic arthritis. He retired from practice and he took up residence at Lawn House, Hanwell, where he could be near the hospital where he had accomplished his most notable work. He grew progressively weaker and found it very difficult to complete several papers and records of his experiences on which he had been working. On March 5, 1867, he suffered a cerebral hemorrhage that proved fatal in a few hours. Although not as well known as Pinel, Conolly, nevertheless, accomplished just as much for the humane treatment of the mentally ill as anyone in the history of psychiatry.